

esophagus and top of the stomach has not yet matured, allowing the stomach contents to come back into the esophagus. If the reflux is severe, your baby may refuse feeds.

What might be done Mild reflux that involves vomiting but no problems with weight gain or pain, usually needs no treatment. More severe reflux will need treatment, usually with medicines to stop or prevent the acid production, and prevent the vomiting. For bottle-fed babies with mild reflux, you may consider an anti-reflux formula.

Pyloric stenosis is most common in first-born male babies and is when the exit at the bottom of the stomach is thickened, meaning that milk cannot leave. It causes so-called projectile vomiting in which a feeding is vomited with so much force that it can land a foot or so away from the baby. The baby is constantly hungry and will demand another feeding immediately after vomiting. Vomiting becomes progressively more severe.

What might be done Pyloric stenosis is diagnosed with a blood test and ultrasound, and treatment is surgical.